



RVMC OB Triage Guidelines

“I’m 7 weeks pregnant and I’m bleeding. Am I having a miscarriage??”

The five major sources of nontraumatic bleeding in early pregnancy are:

- 1. Ectopic pregnancy
- 2. Early pregnancy loss
- 3. Threatened abortion (i.e., vaginal bleeding with evidence of embryonic or fetal viability, such as fetal cardiac activity)
- 4. Physiologic (related to implantation of the pregnancy)
- 5. Cervical, vaginal, or uterine pathology (e.g., polyps, inflammation/infection, gestational trophoblastic disease)

An important goal in the evaluation of patients with bleeding in early pregnancy is to exclude ectopic pregnancy, as rupture can result in severe hemorrhage and death.

Ectopic Pregnancy

Ectopic pregnancy is much less common (prevalence: 2% of pregnancies), but it is the most serious etiology of first-trimester bleeding.

About half of all women who have ectopic pregnancy do not have known risk factors.

Unstable Patients

Patients requiring emergent evaluation:

- Is the patient passing blood clots, or is the blood soaking through her clothes (soaking more than 1 large pad every hour for >2-3 hours?
- Do they feel lightheaded or dizzy/history of syncope?
- Do they have significant pelvic pain or cramping?
- Have they passed any tissue?

If the patient answers “yes” to any of these questions, then ectopic pregnancy and early pregnancy loss are much more likely diagnoses, and the patient needs ED evaluation.

Stable Patients

Table 1. Risk Factors for Ectopic Pregnancy

Risk factor	Odds ratio	95% confidence interval
Previous tubal surgery ⁵	21.0	9.3 to 47
Sterilization ⁶	9.3	4.9 to 18
Previous ectopic pregnancy ⁵	8.3	6.0 to 11.5
In utero exposure to diethylstilbestrol ⁵	5.6	2.4 to 13
Current use of intrauterine device ^{7*}	5.0	1.1 to 2.8
History of pelvic inflammatory disease ⁸	3.4	2.4 to 5.0
Infertility for two years or longer ⁸	2.7	1.8 to 4.2
Advanced maternal age ⁸		
≥ 40 years	2.9	1.4 to 6.1
35 to 39 years	1.4	1.0 to 2.0
Smoking ⁸		
≥ 20 cigarettes per day	3.9	2.6 to 5.9
10 to 19 cigarettes per day	3.1	2.2 to 4.3
1 to 9 cigarettes per day	1.7	1.2 to 2.4
Past smoker	1.5	1.1 to 2.0

*—Refers to situations of intrauterine device failure resulting in pregnancy. Information from references 5 through 8.

Note. From “Diagnosis and management of ectopic pregnancy,” by J. H. Barash, E. M. Buchanan, and C. Hillson, 2014, in *American Family Physician*, 90(1), 34-40.

Patients with Pregnancy <i>Not</i> Confirmed by Ultrasound	Patients with Pregnancy Confirmed by Ultrasound
And any level of abdominal/pelvic pain: • Need to exclude ectopic • ED evaluation is the safest option to ensure a ruptured ectopic pregnancy is neither delayed nor missed. And no abdominal/pelvic pain: • Precautions for heavy bleeding/cramping/pain • For patients with <i>no</i> pain and minimal bleeding, either advise patient to contact OB/GYN in their prenatal services area to arrange appropriate evaluation, <i>OR</i>: • You may choose to consult with OB/GYN On Call in the prenatal service area.	• Reassurance • Precautions for heavy bleeding/cramping • Review Rh status • If bleeding is mild-moderate and occurring >5 days, we recommend you discuss with OB/ GYN On Call in the patient’s prenatal services area to facilitate appropriate evaluation. • If bleeding is mild-moderate and =/< 5 days, advise patient to arrange follow-up with OB/ GYN in their prenatal service area.

Find On Call Physicians in Your Patient’s Service Area

KAISER PERMANENTE. SCAL On Call

Department On Call Physician Directory Help/Info

Select Medical Center

Select Medical Center First

Add SCAL On Call to Your Favorites

<https://scaloncall.kp.org/>

References

Barash, J. H., Buchanan, E. M., & Hillson, C. (2014). Diagnosis and management of ectopic pregnancy. *American Family Physician*, 90(1), 34-40. <https://pubmed.ncbi.nlm.nih.gov/25077500/>

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